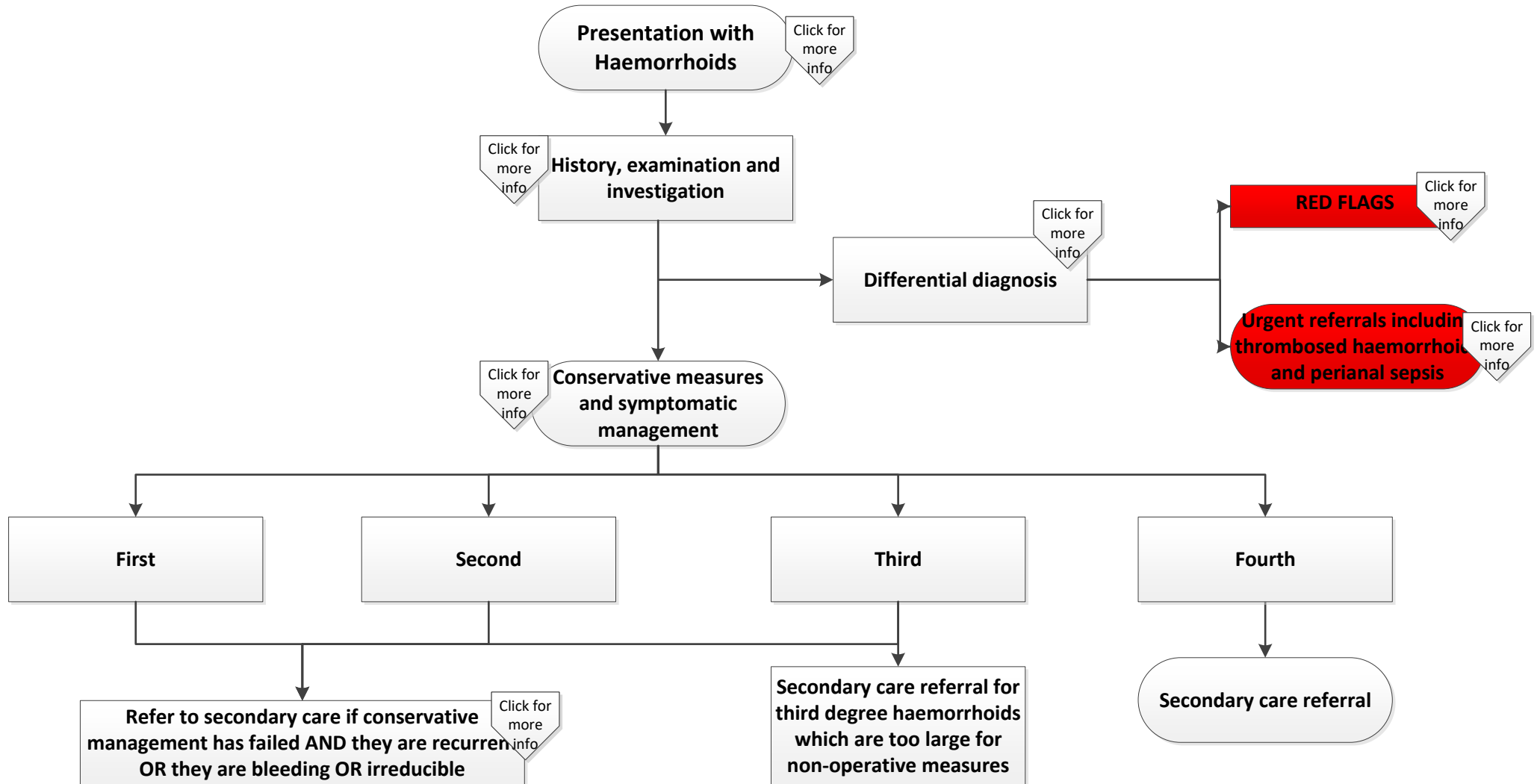


Management of Haemorrhoids



Presentation with Haemorrhoids

Haemorrhoids are a common condition affecting a third of the general population. Peak incidence of haemorrhoids is between ages 45 to 65 years. Symptomatic haemorrhoids increase in prevalence in women during pregnancy and postpartum periods, but subsequently resolve. Most haemorrhoids can be treated in primary care with dietary advice and avoidance of straining, and do not benefit from surgical intervention.

Symptoms

- A person may be asymptomatic, or there may be a history of intermittent symptoms lasting from a few days to a few weeks. In a symptomatic person, it may be difficult to determine whether symptoms are due to internal or external haemorrhoids, especially since people can have internal and external haemorrhoids at the same time.
- Bright-red, painless rectal bleeding with defecation is the most common symptom. It may be streaks on the toilet paper or blood dripping into the toilet. Blood may coat stools but is not mixed in.
- Anal itching and irritation may occur with prolapsing internal haemorrhoids (from prolapse-associated moisture, mucus secretion, and/or faecal incontinence) and with external haemorrhoids (from difficulty with perianal hygiene as a result of skin tags or extensive external haemorrhoids).
- A feeling of rectal fullness, discomfort or of incomplete evacuation on bowel movements may be present if prolapse occurs with straining and in people with large haemorrhoids.
- Prolapsed haemorrhoids may present with a history of a lump at the anal verge.
- Soiling due to mucous discharge or impaired continence may also be experienced.
- Pain is rarely felt with internal haemorrhoids unless the haemorrhoid prolapses and becomes strangulated, resulting in intense pain. External haemorrhoids are usually painless unless thrombosis occurs (the haemorrhoidal tissue becomes engorged and forms clots), resulting in an acutely painful mass in the rectum.
- Strangulated haemorrhoids may thrombose which is intensely painful.
- External haemorrhoids do not usually cause symptoms unless thrombosis occurs causing acute severe pain and a visible/ palpable perianal lump.

History, examination and investigation

History alone does not provide a diagnosis of haemorrhoids.

- Ask about family history of anal or colorectal cancer
- Ask about the presence of predisposing factors for haemorrhoids

Physical examination including digital rectal examination and proctoscopy is a minimum requirement to confirm suspected diagnosis of haemorrhoids and to exclude other anorectal pathology.

All cases of haemorrhoids should be investigated for anaemia and other red flag signs and symptoms. If positive, these cases should be referred on appropriately under the two week wait criteria.

Signs on examination

External examination

- Non-prolapsed internal haemorrhoids are not evident on external examination and are difficult to feel on digital rectal examination.
- Local perineal irritation may be seen if chronic mucous discharge is present.
- Asking the patient to strain may allow haemorrhoids to become visible at the anal verge. They appear as soft, bluish, bulging vessels covered by mucosa.
- If external haemorrhoids are asymptomatic, a bluish bulging of the blood vessel beneath the skin may be seen.
- Acutely thrombosed external haemorrhoids are seen as purple, swollen, tense tender subcutaneous perianal lumps.
- Check for the presence of skin tags or other anal pathology, such as fissures, fistulas, or perianal masses

Digital rectal examination (if this is possible without causing pain) It is essential to carry out digital rectal examination even though internal haemorrhoids will not be palpable. Other pathology needs to be excluded.

A rigid anoscope, proctoscope, or rectoscope can be used to make a diagnosis of haemorrhoids, classify their severity, and exclude sinister pathology. Where the facilities or expertise are not available to perform this investigation in primary care, the person may need to be referred for assessment.

Classification

Haemorrhoids can be classified as internal or external relating to their position of origin either above or below the dentate line. External haemorrhoids originate below the dentate line and are covered by modified squamous epithelium (anoderm), which is richly innervated with pain fibres. External haemorrhoids can therefore be itchy and painful. Internal haemorrhoids arise above the dentate line and are covered by columnar epithelium, which have no pain fibres. Internal haemorrhoids are therefore not sensitive to touch, temperature, or pain (unless they become strangulated).

Haemorrhoids are graded into 4 groups by the Goligher classification defined by their degree of prolapse.

- 1st degree haemorrhoids: May bleed but do not prolapse
- 2nd degree haemorrhoids: Prolapse but reduce spontaneously
- 3rd degree haemorrhoids: Prolapse but reduced manually
- 4th degree haemorrhoids: Permanently prolapsed and irreducible



Red Flags

Refer using the suspected cancer pathway referral (for an appointment within 2 weeks) if anal or colorectal cancer is suspected

- Associated unexplained weight loss
- Anal mass or anal ulceration
- Anaemia
- Rectal mass
- Change in bowel habit

For more information regarding Red flag symptoms and action: [Symptoms suggestive of gastrointestinal tract \(lower\) cancers | Diagnosis | Gastrointestinal tract \(lower\) cancers - recognition and referral | CKS | NICE](#)

Differential diagnosis

Please refer to an appropriate specialist for ongoing management for other diagnoses using clinical judgement to determine the urgency.

Other diagnoses to consider:

- Anal carcinoma – Please see [‘Red flags’](#)
- Colorectal cancer – Please see [‘Red flags’](#)
- Inflammatory bowel disease: Crohn's disease, ulcerative colitis
- Rectal prolapse: a protruding mass and possible faecal incontinence.
- Adenomatous polyps: Bleeding, diarrhoea, and constipation may be present. Rarely a rectal mass may be palpable.
- Anal fissure
- Condylomata acuminata (genital warts): may resemble skin tags and bleed or cause irritation.
- Perianal abscess: which can cause perianal pain, swelling, fever, marked tenderness, induration, redness, and fluctuation.
- Analrectal fistula: may present with anal discharge, itch, intermittent swelling, bleeding, and pain. An external opening may be seen and a tract or cord may be palpable on digital rectal examination. Induration laterally or posteriorly suggests deep perianal or ischiorectal extension.
- Anorectal melanoma — there may be rectal bleeding, perianal pain, and/or a perianal mass. Please see [‘Red flags’](#)
- Diverticular disease
- Portal hypertension — can cause varices of the anal canal which are distinct from haemorrhoids.
- Sexually transmitted infections (gonorrhoea, syphilis, or chancroid)
- Pruritus ani and associated causes - e.g., threadworms, contact dermatitis



Consider urgent referral or admission

Consider admitting people with:

1. Extremely painful, acutely thrombosed external haemorrhoids who present within 72 hours of onset (reduction or excision may be needed).
2. Internal haemorrhoids which have prolapsed and become swollen, incarcerated, and thrombosed (haemorrhoidectomy may be needed).
3. Perianal sepsis (a rare but life-threatening complication).

Thrombosed haemorrhoids

Thrombosed haemorrhoids are extremely painful.

- Consider admission for those presenting early, as some advocate excision under local anaesthetic. Incision and drainage of the clot relieves pain but the thrombosis often recurs and there may be persistent bleeding.
- Conservative treatment includes analgesia, ice packs and stool softeners. A topical calcium antagonist may help to relieve pain.

If managed conservatively, symptoms usually settle within 10-14 days.



Conservative management

For first-, second-, and third-degree haemorrhoids, dietary management consisting of adequate fluid and fibre intake remains the treatment of choice. In addition, bulk forming laxatives (ispaghula), non-opioid analgesia and topical haemorrhoid preparations can be prescribed for symptomatic relief.

- **Ensure stools are soft and easy to pass:**

Bulk forming laxatives can be prescribed for constipation

Advise adequate dietary fibre intake by eating a balanced diet containing whole grains, fruits, and vegetables; this should be done gradually to minimize flatulence and bloating.

Advise that adequate fluid intake is particularly important with an increased fibre diet to maintain soft, well-lubricated stools and to prevent intestinal obstruction.

For advice on constipation please see [Constipation | Health topics A to Z | CKS | NIC](#)

2. Lifestyle advise to aid healing of the haemorrhoid:

Advise on the importance of correct anal hygiene. The anal region should be kept clean and dry to aid healing and reduce irritation and itching. Recommend careful perianal cleansing and to pat (rather than rub) the area dry.

Advise against 'stool withholding' and undue straining during bowel movements, both of which can worsen the condition.

3. Pain and symptom relief:

Simple analgesia - for example, paracetamol.

Avoid constipating opioid analgesics (including codeine). Avoid nonsteroidal anti-inflammatory drugs (NSAIDs) if rectal bleeding is present.

4. Topical therapies: There is no evidence that any topical haemorrhoidal preparations is more effective than another. The choice of preparation should therefore be based on the risk of adverse effects and the person's symptoms and preference. Generally, haemorrhoidal preparations should be used in the morning and night, and after a bowel movement. [Topical haemorrhoidal preparations | Prescribing information | Haemorrhoids | CKS | NICE](#)

- Preparations containing **mild astringents or lubricants** relieve local irritation and are less likely to cause skin sensitization.
- Preparations containing **local anaesthetics** may alleviate pain, burning, and itching, but can cause sensitization of the anal skin. Lidocaine is the preferred topical anaesthetic because others, including tetracaine, cinchocaine (dibucaine), and pramocaine (pramoxine), are more irritant. Anaesthetic-containing preparations should only be used for a few days because they may cause sensitization of the anal skin.
- Preparations containing **corticosteroids** may reduce inflammation and pain, but prolonged use may lead to skin atrophy, contact dermatitis, and skin sensitization. Local infection (for example herpes simplex infection or perianal thrush) must be excluded before use. Corticosteroid-containing preparations should be used for no longer than 7 days because prolonged use may lead to skin atrophy, contact dermatitis, and skin sensitization. Excessive application should be avoided, as systematic absorption can occur through the rectal mucosa.

- No topical haemorrhoidal preparations are licensed for use during pregnancy or breastfeeding. However, the potential risk of harms to the pregnant woman and/or fetus is likely to be less with simple, soothing products than with those containing corticosteroid and/or local anaesthetic (which are generally not recommended by the manufacturers for use during pregnancy due to inadequate safety evidence).

5. Additional advice:

- Good perianal hygiene may be helpful in providing symptomatic relief and preventing perineal dermatitis. Moistened towelettes or baby wipes can be used to clean the perianal area. The area should then be patted dry.
- Advise the person that when the haemorrhoid has healed, they should continue with dietary and lifestyle measures to reduce the risk of recurrence.
- Straining at stool should be avoided as it can make symptoms worse.

Refer to secondary care if conservative treatment has failed and symptoms are severe

As a non-priority condition, haemorrhoidectomy will only be commissioned if conservative treatment has failed and one of the following three criteria is present:

1. Haemorrhoids are recurrent;
2. Haemorrhoids are causing persistent bleeding;
3. Haemorrhoids cannot be reduced

Surgical options:

Surgical treatment should only be considered for those that do not respond to the non-operative measures or if the haemorrhoids are more severe, specifically:

- Recurrent grade 3 or grade 4 combined internal/external haemorrhoids with persistent pain or bleeding; or
 - Irreducible and large external haemorrhoids.
 - In cases where there is significant rectal bleeding the patient should be examined internally by a specialist.
1. Haemorrhoidectomy: Only symptomatic haemorrhoids are excised as this conserves the sensitive anoderm for continence.
 2. Stapled haemorrhoidectomy: A circular stapling gun is used to excise a doughnut of mucosa from the upper anal canal and lift the haemorrhoidal cushions back within the canal. Although termed haemorrhoidectomy, it is more accurately termed a haemorrhoidopexy as the haemorrhoids are not excised but relocated within the anal canal.
 3. Haemorrhoidal artery ligation: Using a proctoscope, the haemorrhoidal arteries are ligated with sutures (above the dentate line) to remove the flow of blood to the haemorrhoids. For larger prolapsing haemorrhoids, an adjunctive mucosal plication procedure is done; the prolapsing mucosa is plicated up to the level of the dentate line where it is fixed by ligation of the plicating sutures (haemorrhoidopexy).

Non Surgical options:

Often haemorrhoids (especially early stage haemorrhoids) can be treated by simple measures such as eating more fibre or drinking more water. If these treatments are unsuccessful many patients will respond to outpatient treatment in the form of banding or perhaps injection.

1. Rubber band ligation: A band is applied to the base of the haemorrhoid. The strangulated haemorrhoid becomes necrotic and sloughs off. The underlying tissue undergoes fixation by fibrotic wound healing. Up to three haemorrhoids can be banded at one visit.

- This is currently the best available outpatient treatment of haemorrhoids. About 2 in 10 people will require a second banding within 6 months for symptom control.
- Minor complications include haemorrhoid thrombosis, band displacement, mild bleeding, and formation of mucosal ulcers.

2. Injection sclerotherapy: Phenol in oil is injected into the submucosa of the rectum, around the pedicles of the haemorrhoids. It induces a fibrotic reaction which obliterates the haemorrhoidal vessels, causing atrophy of the haemorrhoids.

- Provides short-term benefit in most people.
- Complications include pelvic infection and erectile dysfunction due to incorrectly sited injections (rare).

3. Infrared coagulation/photocoagulation: This involves using infrared energy to produce an area of submucosal fibrosis leading to mucosal fixation and a reduction in the tendency to prolapse.

- It may be as effective as rubber band ligation and injection sclerotherapy in the treatment of first degree and second degree haemorrhoids.

4. Bipolar diathermy and direct-current electrotherapy: Causes coagulation and fibrosis after local application of heat.

- Success rates are similar to those of infrared coagulation, and complication rates are low.